

How Norwegian politicians talk about access to healthcare

Access to healthcare is something Norwegian politicians talk about regularly. Still, access does not always mean the same thing. It can refer to waiting times, the distance to a hospital, access to a regular general practitioner, continuity between services, patient rights, or the cost of receiving treatment.

I wanted to look more closely at which dimensions of healthcare access appear in speeches from the Norwegian Parliament. More specifically, I was interested in three questions. Which dimensions receive the most attention? Do the political blocs devote different shares of their speeches to access? And do they emphasize different parts of the issue?

The larger data collection consists of 15,012 parliamentary speeches published between July 28, 2025, and July 28, 2026. The figures presented here focus on speeches held between January 6 and June 19, 2026. The speeches were collected from the Norwegian Parliament's open data services using the R package *stortingscrape* by Martin Søyland.

Instead of manually coding every speech or using a large language model, I created a predefined list of Norwegian keywords. These keywords were used to identify six dimensions of healthcare access: geography and local services, primary care and continuity, waiting times and capacity, staff shortages and recruitment, patient rights and choice, and financial barriers. A speech could be included in more than one category.

Geography and local services receive the most attention

The analysis identified 562 speeches that referred to at least one dimension of healthcare access between January 6 and June 19, 2026, as shown in Figure 1. Because a speech could be assigned to more than one category, the category totals exceed the number of unique access-related speeches.

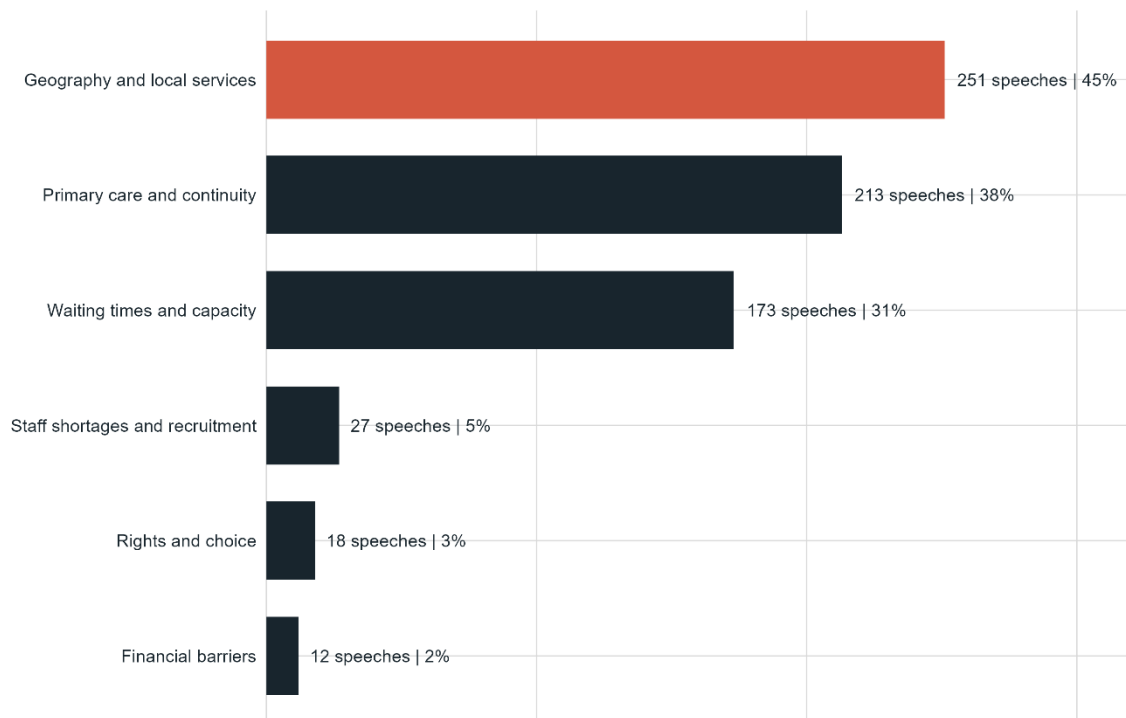
Geography and local services received the most attention. This dimension appeared in 251 speeches, corresponding to 45 percent of all access-related speeches. Primary care and continuity appeared in 213 speeches, or 38 percent, while waiting times and capacity appeared in 173 speeches, or 31 percent.

The remaining dimensions appeared much less frequently. Staff shortages and recruitment were identified in 27 speeches, rights and choice in 18, and financial barriers in 12.

These results show that parliamentary discussions of healthcare access are not limited to waiting lists. Politicians also devote considerable attention to where services are located, whether local services remain available, and whether patients experience continuity between different parts of the healthcare system.

Figure 1

Geography and local services were the most common way politicians discussed access to healthcare, followed by primary care and continuity and by waiting times and capacity. One speech could be included in more than one category.



The blocs give access fairly similar levels of attention

Figure 2 compares the share of speeches from each political bloc that referred to at least one dimension of healthcare access. Government speakers were excluded because they speak from a different institutional position than ordinary parliamentary representatives.

Healthcare access appeared in 5.9 percent of speeches from the left, corresponding to 115 of 1,964 speeches. It appeared in 5.2 percent of speeches from the right, or 178 of 3,428 speeches, and in 4.3 percent of speeches from the center, or 84 of 1,958 speeches.

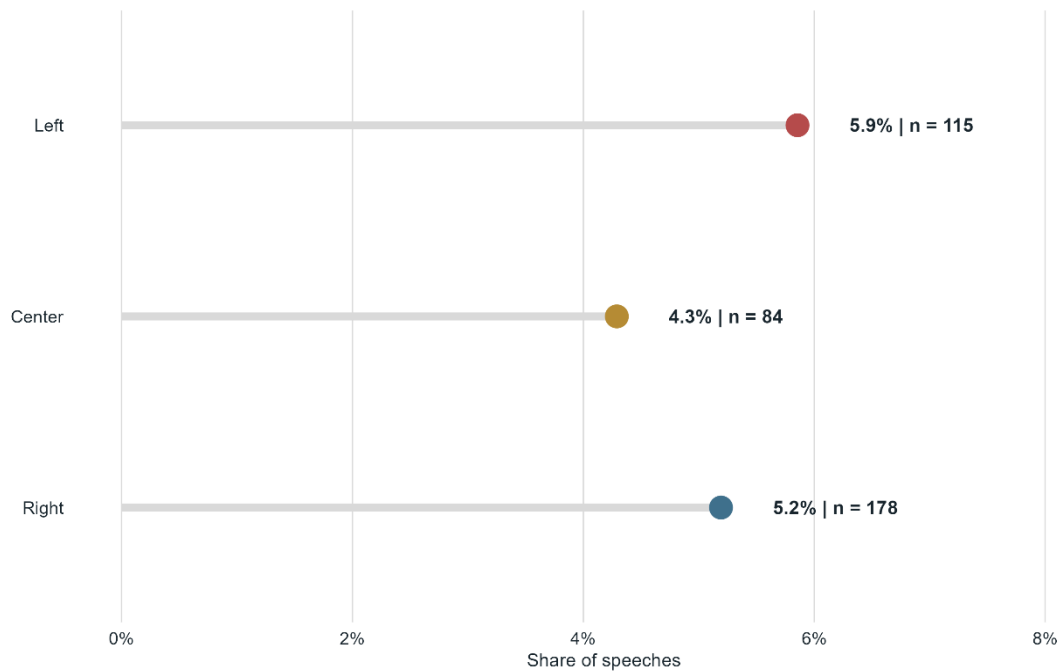
The left therefore devoted the largest share of its speeches to healthcare access, although the difference between the left and the right was modest. The right had the largest number of access-related speeches because it also accounted for a larger overall volume of parliamentary speeches.

It is therefore useful to report both numbers and percentages. The number of speeches reflects the overall volume of communication, while the percentage shows how much attention access receives within each bloc's total parliamentary activity.

The results do not support a simple interpretation in which one political side discusses access while another ignores it. All three blocs devoted broadly similar shares of their speeches to the issue.

Figure 2

The three political blocs devoted fairly similar shares of their speeches to healthcare access. The left had the highest share, while the right had the largest number of access-related speeches.



The blocs emphasize different barriers

The clearest political differences concerned which dimensions of healthcare access each bloc emphasized, as shown in Figure 3.

The right gave relatively more attention to waiting times and capacity. The share of its access-related speeches referring to this dimension was 7.5 percentage points above the overall distribution in the material. Geography and local services, by contrast, were 8.3 percentage points below the overall distribution.

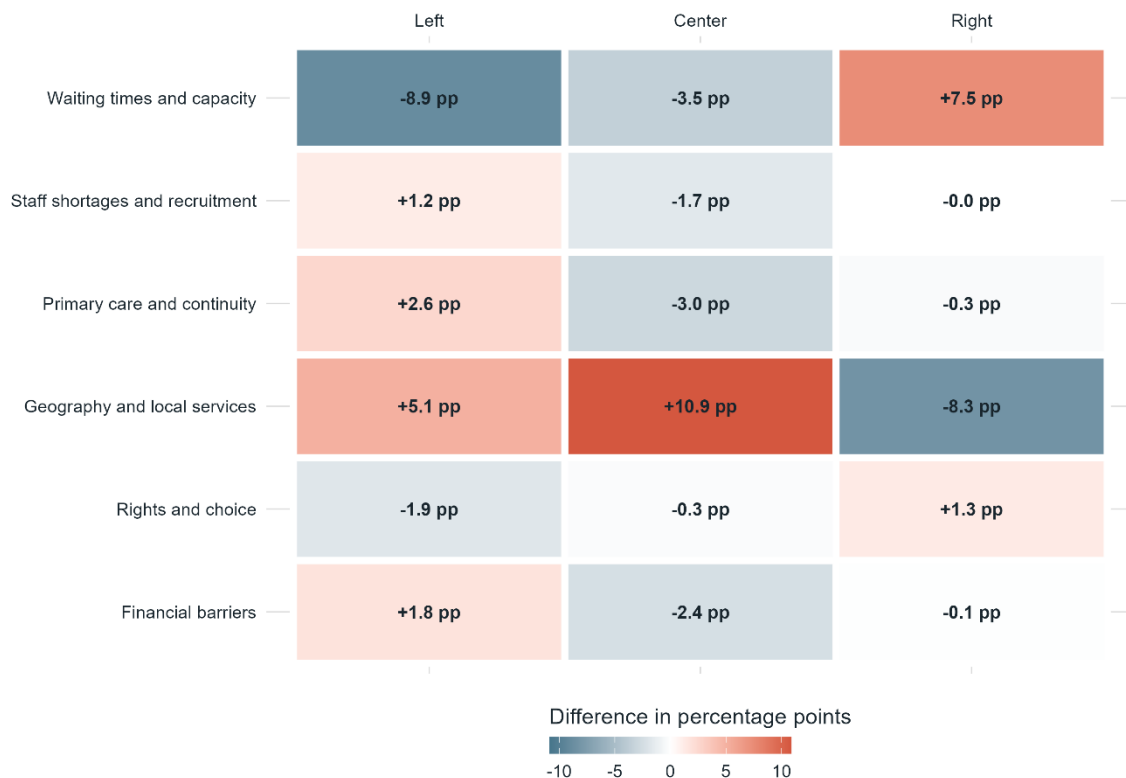
The left and the center displayed a different pattern. Geography and local services were 5.1 percentage points above the overall distribution on the left and 10.9 percentage points above it in the center. The left also gave somewhat more attention to primary care and continuity, which was 2.6 percentage points above the overall distribution. Waiting times and capacity were 8.9 percentage points below the overall distribution on the left and 3.5 percentage points below it in the center.

These findings suggest that political differences emerge before the parties begin presenting specific solutions. The blocs do not always define access to healthcare as the same type of problem.

When access is described primarily as a question of waiting times, attention is likely to turn toward capacity, efficiency, and organization. When it is described as a geographical problem, distance, local services, and decentralization become more important. When continuity is emphasized, the focus moves toward general practitioners, primary care, and coordination between services.

Figure 3

The blocs emphasized different sides of healthcare access. The right focused more on waiting times and capacity, while the left and center placed more emphasis on geography and local services.



Several barriers appear in the same speeches

Figure 4 examined which dimensions of healthcare access appeared together.

The most common combination was geography and local services together with primary care and continuity. These dimensions appeared together in 48 speeches, corresponding to 8.5 percent of all access-related speeches.

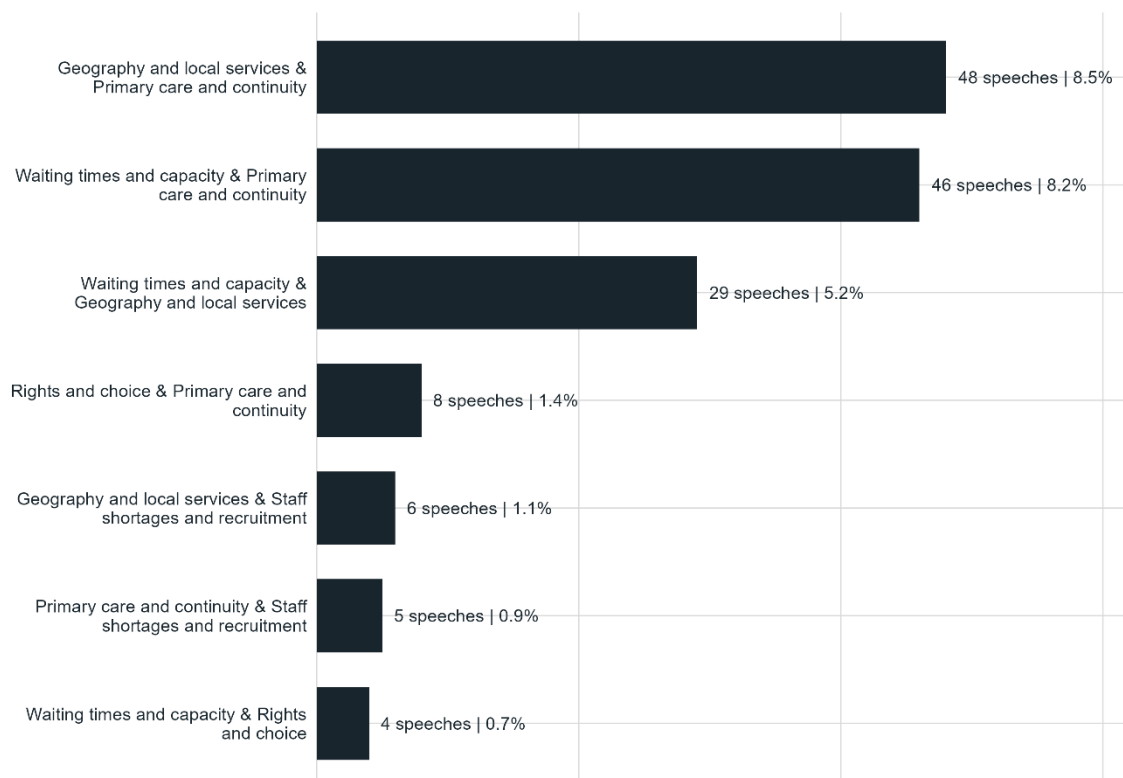
Waiting times and capacity appeared together with primary care and continuity in 46 speeches, or 8.2 percent. Waiting times and capacity appeared together with geography and local services in 29 speeches, or 5.2 percent.

Less common combinations included rights and choice together with primary care and continuity, which appeared in eight speeches, and geography and local services together with staff shortages and recruitment, which appeared in six speeches.

The results show that politicians frequently discuss healthcare access as a combination of barriers rather than as one isolated problem. A patient may, for example, live far from a service while also experiencing limited continuity or long waiting times.

Figure 4

Healthcare access was often discussed as a combination of problems. Geography and local services most commonly appeared together with primary care and continuity.



A few sitting days account for many of the speeches

Healthcare access was not discussed evenly throughout the period. June 9 had the largest number of access-related speeches, with 51. February 24 followed with 37 speeches, while April 23 had 36 and April 22 had 34.

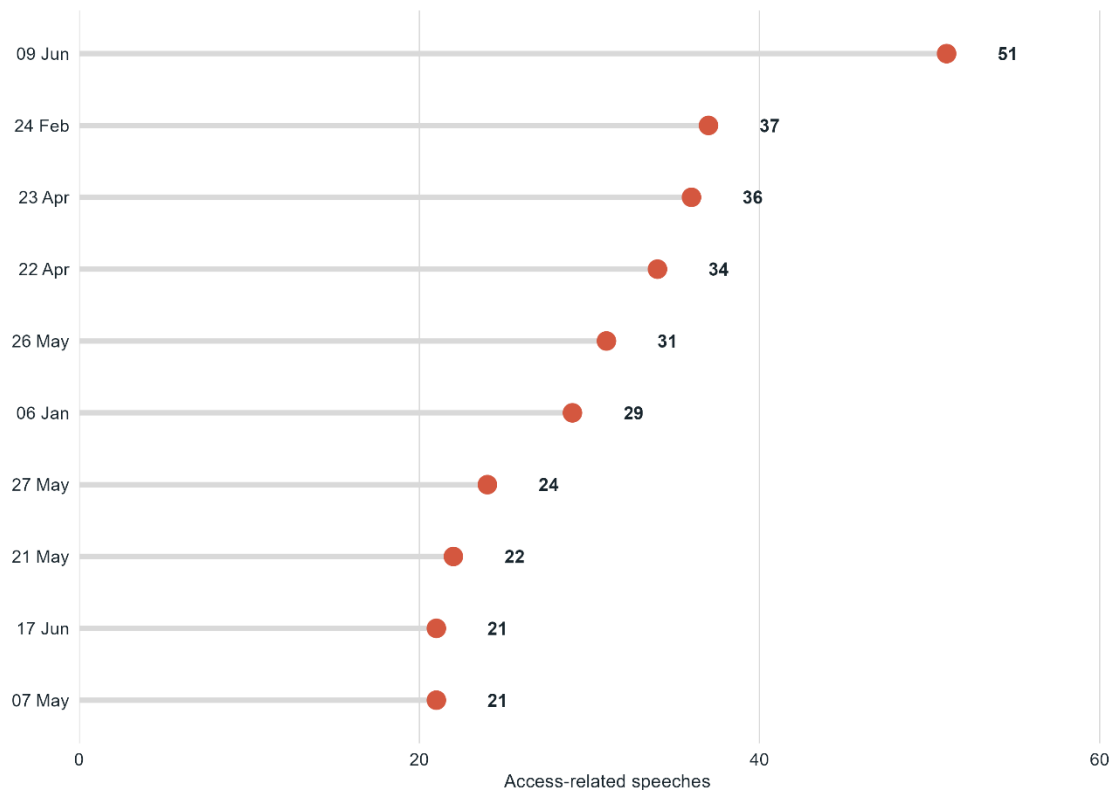
The ten most active sitting days contained a total of 306 access-related speeches. This means that a substantial part of the observed attention was concentrated on a relatively small number of parliamentary sitting days.

This concentration does not necessarily mean that healthcare access suddenly became more politically important on these dates. Parliamentary speech activity is shaped by the agenda. A major healthcare debate, question time, or committee recommendation may generate many speeches on the same day.

The peaks should therefore be interpreted in relation to the parliamentary business being discussed on each date.

Figure 5

Attention to healthcare access was concentrated on a small number of parliamentary sitting days, with June 9 standing out as the busiest day in the period.



What the analysis shows

The analysis demonstrates that access to healthcare does not have one fixed meaning in Norwegian parliamentary debate.

The political blocs do not differ dramatically in how frequently they discuss access. The more important difference concerns what they emphasize. The right gives relatively more attention to waiting times and capacity. The left and the center give more attention to geography and local services, while the left also places somewhat greater emphasis on primary care and continuity.

Political communication is therefore not only about proposing different solutions. It also contributes to defining what the principal problem is in the first place.

This is a descriptive analysis. It measures which terms and dimensions appear in parliamentary speeches, but it does not measure the actual quality or availability of healthcare services. The keyword-based approach also has limitations. Some relevant speeches may use terms that are not included in the dictionary, while individual keywords may occasionally appear in another context. The results should therefore be interpreted as a systematic overview of patterns in parliamentary language rather than a complete classification of every debate about healthcare access.